

Mandatory Reporters in Cases of Neglect, Abuse, and Exploitation: A U.S. Legal, Empirical, and Policy Report

TL;DR

- A “mandatory reporter” is a person whom statute requires—on pain of criminal misdemeanor, professional discipline, and (in some states) civil liability—to report a reasonable suspicion of child abuse or neglect, or abuse, neglect, or exploitation of an elder or vulnerable adult, to a designated agency such as Child Protective Services (CPS), Adult Protective Services (APS), or law enforcement, with the duty and immunity structures set by state statutes implementing federal frameworks like the Child Abuse Prevention and Treatment Act (CAPTA, 42 U.S.C. §§ 5101 et seq.) and the Elder Justice Act (EJA, 42 U.S.C. § 1397j et seq.).
- The system has produced landmark accountability—*Landeros v. Flood* (Cal. 1976) establishing civil liability for non-reporting physicians, and the Penn State/Sandusky prosecutions of Spanier, Curley, and Schultz—but also serious unintended harms documented in peer-reviewed research: 37.4% of all U.S. children and 53.0% of Black children experience a CPS investigation by age 18 (Kim, Wildeman, Jonson-Reid & Drake, *American Journal of Public Health*, 2017, 107(2):274–280), [Semantic Scholar](#) [nih](#) and 35% of the 2,462 intimate-partner-violence survivors surveyed by Lippy et al. (*Journal of Family Violence*, 35(3):255–267, 2020) “did not ask for help from at least one person because of mandatory reporting laws.” [Casey Family Programs](#)
- Reform should be staged: invest in evidence-based training (e.g., the iLookOut RCT in Mathews et al., *PLoS ONE*, 2017, showed knowledge gains with Cohen’s $d = 0.95$), [plos](#) narrow neglect definitions to distinguish poverty from maltreatment, restore survivor autonomy in Title IX and domestic-violence contexts via “mandatory supporting” models, and preserve criminal liability for clear failures to report serious child or elder abuse.

Key Findings

1. **Definition and legal architecture.** Mandatory reporters are professionals (and, in 18 jurisdictions plus Puerto Rico, all adults) whose state statute imposes a duty to report a

“reasonable suspicion” of abuse, neglect, or exploitation. CAPTA (originally Pub. L. No. 93-247, 1974; codified at 42 U.S.C. §§ 5101–5119) conditions federal child-welfare grants on states maintaining such laws; the EJA (Pub. L. No. 111-148, § 6703, 2010; 42 U.S.C. § 1397j et seq.) imposes a federal duty on “covered individuals” in long-term-care facilities to report reasonable suspicion of crimes against residents.

2. **Failure-to-report liability runs criminal and civil.** California criminalizes non-reporting as a misdemeanor (Cal. Penal Code § 11166(c)). *Landeros v. Flood*, 17 Cal. 3d 399 (1976), allowed civil tort recovery under a negligence-per-se theory. The Pennsylvania prosecution of Penn State officials Graham Spanier, Tim Curley, and Gary Schultz showed that even university presidents can face jail time for failing to escalate suspected abuse.
3. **Reporting duties almost always override professional privilege**—with the conspicuous, contested exception of clergy-penitent privilege. The Child Welfare Information Gateway lists 29 states that name clergy as mandatory reporters; according to an Associated Press review (Sept. 28, 2022), “Thirty-three states, including Washington, Idaho and Oregon, exempt clergy from laws requiring professionals such as teachers and doctors to report information about alleged child abuse to police or child welfare officials if the church deems the information privileged.”
[Northwest Public Broadcasting](#) [Spectrum News 1](#) The boundary is under fresh litigation, including *Etienne v. Ferguson*, No. 2:25-cv-00766 (W.D. Wash. 2025), over Washington’s SB 5375.
4. **The Constitution does not require state actors to protect a known-at-risk child** from private violence (*DeShaney v. Winnebago County Dep’t of Social Services*, 489 U.S. 189 (1989)), but it does protect patients from warrantless drug testing aimed at law-enforcement reporting (*Ferguson v. City of Charleston*, 532 U.S. 67 (2001)).
5. **Reporting produces large racial and socioeconomic disparities.** Kim, Wildeman, Jonson-Reid & Drake (*AJPH*, 2017) estimated that 37.4% of U.S. children—and 53.0% of Black children—experience a CPS investigation by age 18. [Semantic Scholar](#) [nih](#) A landmark *NEJM* study in Pinellas County, Florida found that Black and white pregnant women [American Civil Liberties Union](#) had nearly identical rates of substance use (14.1% vs. 15.4%) [American Civil Liberties Union](#) but Black women were reported “at approximately 10 times the rate for white women [American Civil Liberties Union](#) (P<0.0001)” [PubMed](#) (Chasnoff, Landress & Barrett, *New England Journal of Medicine*, 322(17):1202–1206, 1990).
6. **Universal mandatory reporting (UMR) does not improve detection of confirmed abuse.** A NCANDS-based study summarized by Casey Family Programs (2021) found that, controlling for child and caregiver characteristics, reports by nonprofessionals were *less* likely to be confirmed in UMR states (6.6%) than in non-UMR states (12.0%).

7. **Title IX universal mandatory reporting in higher education is opposed by most survivors and most empirical researchers.** Holland, Cortina & Freyd (*PNAS*, 2022) and the National Academies (2018) recommend “mandatory supporting” instead.
8. **Training is necessary, common, and demonstrably effective when well-designed.** The iLookOut RCT (Mathews et al., *PLoS ONE*, 2017; n = 741) demonstrated a large knowledge effect (Cohen’s d = 0.95); a real-world replication in n = 11,065 providers confirmed generalizability (Yang et al., *PLoS ONE*, 2020). But state-sponsored curricula are uneven: Baker, LeBlanc, Adebayo & Mathews (*Child Abuse & Neglect*, 113:104932, 2021) found that “only about one fourth [of state curricula] included a definition, indicators, and examples of each type of maltreatment.” Cambridge Core

Details

1. What is a Mandatory Reporter?

A “mandatory reporter” (sometimes “mandated reporter”) is a person whom state statute legally compels to make a report to a designated public agency—typically a CPS or APS hotline or law enforcement—whenever, within the scope of their profession or duties, they form a “reasonable suspicion” (or, in some states, “reasonable cause to believe” or “know or suspect”) that a child or vulnerable adult has been abused, neglected, or exploited. The duty is personal: it cannot be discharged by reporting up an internal chain of command Michigan Department of Health & H Darkness to Light (see, e.g., Mich. Comp. Laws § 722.623; Cal. Penal Code § 11166(i)(3)).

Federal floor. Although there is no federal child-abuse reporting statute that directly binds individuals, the **Child Abuse Prevention and Treatment Act**, originally enacted in 1974 (Pub. L. No. 93-247) and codified principally at 42 U.S.C. §§ 5101–5119, conditions state eligibility for federal child-welfare formula grants on a state plan that includes mandatory reporting laws, immunity for good-faith reporters, and training for mandated reporters (42 U.S.C. § 5106a(b)(2)(B)). CAPTA defines “child abuse and neglect” as “any recent act or failure to act on the part of a parent or caretaker which results in death, serious physical or emotional harm, sexual abuse or exploitation, or an act or failure to act which presents an imminent risk of serious harm” Mandated Reporter (42 U.S.C. § 5101 note). The Victims of Child Abuse Act Reauthorization of 2018 (Pub. L. No. 115-424) added an immunity provision for good-faith reporters under federal CAPTA programs. Wikipedia

For older and dependent adults, the **Elder Justice Act** (enacted as § 6703 of the Patient Protection and Affordable Care Act, Pub. L. No. 111-148, 2010; codified at 42 U.S.C. § 1397j et seq.) directly requires “covered individuals”—essentially all owners, operators,

employees, managers, agents, and contractors of long-term-care facilities receiving at least \$10,000 in federal funds—to report any “reasonable suspicion of a crime” against a resident to the HHS state-survey agency and at least one local law-enforcement agency, within two hours if the suspicion involves serious bodily injury, otherwise within 24 hours [FindLaw](#) (42 U.S.C. § 1397j-1). Civil monetary penalties run up to \$200,000 for failure to report and \$300,000 where the failure exacerbates harm; retaliation against reporters subjects facilities to penalties up to \$200,000 and possible exclusion from Medicare/Medicaid.

[Siegel Law](#)

[Mandatedreportertraining](#)

Who is covered in the states. Coverage varies. As of 2023, the Child Welfare Information Gateway counted 18 states plus Puerto Rico as “universal mandatory reporter” jurisdictions (any adult must report), with New Jersey and Wyoming being canonical examples

[Mandated Reporter](#)

(*Mandatory Reporting of Child Abuse and Neglect*, U.S. Dep’t of Health and Human Services, Children’s Bureau, 2023). Most states enumerate professional categories—typically educators, healthcare practitioners, mental health professionals, social workers, child-care providers, law enforcement officers, coroners, clergy (in the 29 states listed by HHS), and increasingly commercial film/photo processors, computer technicians, and bank employees in financial-exploitation contexts [Mandated Reporter](#) (see, e.g., Cal. Penal Code § 11165.7; N.Y. Soc. Serv. Law § 413; Cal. Welf. & Inst. Code § 15630).

The “reasonable suspicion” standard. Forty-five states use “reasonable” belief or suspicion as the triggering threshold. [Psychiatry Online](#) California codifies the leading definition: reasonable suspicion is suspicion that is “objectively reasonable for a person to entertain [Thurmanarnold](#) ... based upon facts that could cause a reasonable person in a like position, drawing, when appropriate, on the person’s training and experience, to suspect child abuse or neglect” [Psychiatry Online](#) (Cal. Penal Code § 11166(a)(1)). It “does not require certainty ... nor ... a specific medical indication of child abuse or neglect.” [FindLaw](#) The reporter is not an investigator; the burden of substantiation lies with CPS or law enforcement. [Mandated Reporter](#)

2. Court Cases Raising Mandatory-Reporter Issues

A. Failure-to-report civil liability: *Landeros v. Flood*, 17 Cal. 3d 399, 551 P.2d 389, 131 Cal. Rptr. 69 (1976). Eleven-month-old Gita Landeros was treated by Dr. A.J. Flood

[Studicata](#)

at the San Jose Hospitals & Health Center for a spiral leg fracture, multiple bruises, and a healing skull fracture— [Studicata](#) classic signs of battered-child syndrome.

Flood neither diagnosed BCS nor reported the injuries [Studicata](#) under then-Penal Code § 11161.5. Returned home, Landeros suffered further beatings, [Studicata](#) including severe head trauma. The California Supreme Court reversed dismissal, holding (per Justice Mosk)

that (i) a physician owes a common-law duty of due care that includes recognizing and reporting BCS where indicated, (ii) violation of the reporting statute establishes negligence per se (now codified at Cal. Evid. Code § 669), [Justia](#) [FindLaw](#) (iii) further beatings by the

parents did not break the chain of proximate causation because such reabuse was foreseeable, [Lawpipe](#) and (iv) the physician-patient privilege does not bar reporting because § 11161.5 specifically conferred immunity. [Justia](#) [California Supreme Court Resource](#); *Landeros* remains the seminal U.S. authority for civil tort liability for failure to report.

B. Distinguishing negligence per se from standard-of-care: *Becker v. Mayo Foundation*, 737 N.W.2d 200 (Minn. 2007). The Minnesota Supreme Court declined to make the reporting statute an independent basis for tort liability, holding instead that a mandatory reporting statute may inform the standard of care in a common-law malpractice claim [American Medical Association](#) but does not itself create a private right of action. *Becker* illustrates the state-by-state split between *Landeros*-style negligence-per-se jurisdictions and those requiring a traditional common-law negligence theory.

C. Criminal failure-to-report and institutional duty: the Penn State officials' prosecutions. Three former Penn State officials—President Graham Spanier, Athletic Director Tim Curley, and Vice President Gary Schultz—were charged with [Wikipedia](#) endangering the welfare of children (18 Pa. Cons. Stat. § 4304) for failing in 2001 to report [NPR](#) or escalate graduate assistant Mike McQueary's [NPR](#) eyewitness account of former defensive coordinator Jerry Sandusky [NPR](#) sexually abusing a boy in a Penn State locker room. Curley and Schultz pleaded guilty to misdemeanor child endangerment; Spanier was convicted by jury in 2017 and sentenced to four to twelve months. [Wikipedia](#) After a federal magistrate (Judge Karoline Mehalchick) [The Philadelphia Inquirer](#) vacated the conviction on ex post facto/due-process grounds—holding that the 2007 amendment expanding § 4304 to cover supervisors of caregivers was applied retroactively—the Third Circuit reinstated the conviction in December 2020, and Spanier served 58 days in 2021. [Wikipedia](#) Sandusky himself was convicted in 2012 [NPR](#) on 45 counts [Wikipedia](#) (*Commonwealth v. Sandusky*, 70 A.3d 886 (Pa. Super. 2013)) and sentenced to 30–60 years.

D. State's duty to protect: *DeShaney v. Winnebago County Department of Social Services*, 489 U.S. 189 (1989). Joshua DeShaney, [Legal Information Institute](#) beaten into permanent brain damage by his father after numerous abuse reports to DSS were inadequately followed up, sued under 42 U.S.C. § 1983. The Supreme Court (Rehnquist, C.J., 6–3) held that the Due Process Clause [Studicata](#) of the Fourteenth Amendment is a limitation on state power, not a guarantee of safety; the state had no constitutional affirmative duty to protect because DeShaney was in his father's, not the state's, custody. Justice Blackmun's dissent ("Poor Joshua!") remains famous. *DeShaney* effectively closed federal § 1983 as a route to hold CPS agencies liable for inaction in most cases; *Castle Rock v. Gonzales*, 545 U.S. 748 (2005), reinforced this line. [Wikipedia](#) Plaintiffs have since relied on state tort law and on narrow exceptions—the "special-relationship" doctrine and the "state-created-danger" doctrine recognized in some circuits.

E. Constitutional limits on coercive reporting: *Ferguson v. City of Charleston*, 532 U.S.

67 (2001). The Medical University of South Carolina, [American Civil Liberties Union](#) in cooperation with the Charleston Police, drug-tested pregnant women [Wikipedia](#) meeting nine criteria and reported positive cocaine results to police, who arrested 30 women—29 of whom were [American Civil Liberties Union](#) Black—for child abuse and drug distribution to a minor. In a 6–3 decision authored by Justice Stevens, the Supreme Court held the testing violated the Fourth Amendment because its “immediate objective [Wikipedia](#) ... was to generate evidence for law enforcement purposes,” [Louisiana State University Law Cent](#) disqualifying it from the “special needs” exception. *Ferguson* is the leading case constraining the use of mandatory reporting as a tool of criminal investigation, especially where racial disparity is acute. The dissent (Scalia, J., joined by Rehnquist and Thomas) emphasized states’ longstanding authority to require healthcare professionals to report criminal evidence—reflecting the unresolved tension between mandatory reporting and Fourth Amendment privacy.

F. Clergy-penitent privilege and mandatory reporting. State law on clergy reporting varies sharply. Per Child Welfare Information Gateway (2023), the 29 states that include clergy as mandated reporters are “Alabama, Arizona, Arkansas, California, Colorado, Connecticut, Georgia, [Ntcumc](#) Hawaii, Illinois, Louisiana, Maine, Massachusetts, Michigan, Minnesota, Mississippi, Missouri, Montana, Nevada, New Hampshire, New Mexico, North Dakota, Ohio, Oregon, Pennsylvania, South Carolina, Vermont, Virginia, West Virginia, [Ntcumc](#) and Wisconsin.” [Capuchin](#) According to the Associated Press (Sept. 28, 2022), “Thirty-three states ... exempt clergy from laws requiring professionals such as teachers and doctors to report information about alleged child abuse to police or child welfare officials if the church deems the information privileged.” [Northwest Public Broadcasting](#) [Spectrum News 1](#) Seven states (NH, NC, OK, RI, TX, WV; TN for child sexual abuse) effectively impose no confessional exemption. [Stateline](#) [MinistryWatch](#) Courts have grappled with the boundary between sacramental confession and other pastoral communications; the Arizona Supreme Court let LDS bishops withhold [Rolling Stone](#) abuse evidence in litigation reported in 2023. [Substack](#) *Etienne v. Ferguson*, No. 2:25-cv-00766 (W.D. Wash. 2025), challenged Washington’s SB 5375 (codified at Wash. Rev. Code § 26.44.030), which eliminated the clergy-penitent exception; [Washington State Standard](#) Washington settled in October [MinistryWatch](#) 2025, restoring the confessional exemption while keeping clergy as mandated reporters in all other contexts. The U.S. Department of Justice intervened on the Catholic priests’ side, asserting Free Exercise concerns.

G. Good-faith immunity for reporters. Every state and the District of Columbia provide some form of statutory immunity for mandatory reporters acting in good faith. CAPTA requires such immunity as a condition of funding (42 U.S.C. § 5106a(b)(2)(B)(vii)), and the 2018 Victims of Child Abuse Act Reauthorization (Pub. L. No. 115-424) added explicit federal immunity. The American Bar Association Commission on Law and Aging maintains a 50-state survey of immunity provisions in APS statutes (ABA, *Immunity for Good Faith*

Reporting: Criteria in Adult Protective Services Laws, 2022). Typical statutes (e.g., Cal. Penal Code § 11172; Mass. Gen. Laws ch. 119, § 51A; Tex. Fam. Code § 261.106) shield reporters from civil and criminal liability (LegalClarity) and often confidentiality of the reporter's identity.

H. The opposite tort: liability for *bad-faith* or negligent reporting. Good-faith immunity is not absolute. Courts have permitted defamation and intentional-infliction claims where reports were made in bad faith or with knowing falsity (see, e.g., *Stecks v. Young*, 38 Cal. App. 4th 365 (1995); *Heinrich v. Conemaugh Valley Mem'l Hosp.*, 648 A.2d 53 (Pa. Super. 1994)).

3. Contested and Problematic Issues

A. Over-reporting, false positives, and racial/socioeconomic disparities. The most comprehensive empirical estimate of cumulative CPS exposure comes from Kim, Wildeman, Jonson-Reid & Drake (*American Journal of Public Health*, 2017, 107(2):274–280): "We estimate that 37.4% of all children experience a child protective services investigation by age 18 years. Consistent with previous literature, we found a higher rate for African American children (53.0%) and the lowest rate for Asians/Pacific Islanders (10.2%)."

(Semantic Scholar) (nih) Cumulative substantiation was 11.8%— (nih) meaning the majority of investigated children are never confirmed as victims. The disparity is not explained by differential maltreatment rates: in Chasnoff, Landress & Barrett's classic *New England Journal of Medicine* study (1990, 322(17):1202–1206), Black and white pregnant women in Pinellas County had nearly identical rates of substance use (14.1% vs. 15.4%), yet Black women were reported "at approximately 10 times the rate for white women ($P < 0.0001$), (American Civil Liberties Union) and poor women (American Medical Association) were more likely than others to be reported." (PubMed) Recent NCANDS-based analyses confirm that medical reports of infants (nih) exposed to prenatal substance use rose sharply over 2010–2019, with Black infants investigated (nih) at higher rates than white infants (Rebbe et al., *Pediatrics/Hospital Pediatrics*, 2023). Krase (*Journal of Public Child Welfare*, 7(4):351–369, 2013) showed that racial disproportionality and disparity in maltreatment reports differ across report sources— (ResearchGate) medical and educational reporters often contribute disproportionately to over-representation of Black children. Dorothy Roberts (Univ. of Pennsylvania) argues in *Torn Apart: How the Child Welfare System Destroys Black Families* (Basic Books, 2022) and "Why Abolition," All Faculty Scholarship No. 2990 (2023), (University of Pennsylvania Carey La) that mandated reporting functions as a "family policing" regime concentrated in poor Black and Indigenous communities. (Dissent Magazine)

B. Chilling effects on help-seeking. Lippy, Jumarali, Nnawulezi, Williams & Burk (*Journal of Family Violence*, 35(3):255–267, 2020), surveying 2,462 intimate-partner-violence survivors via the National Domestic Violence Hotline, (ScienceGate) found that "35% of respondents did not ask for help from at least one person because of mandatory reporting

laws"; [Casey Family Programs](#) provider warnings about [Springer](#) mandatory reporting often reduced survivors' ability to receive sought support; and reports, when triggered, made the situation worse for most survivors—particularly for trans, gender-non-conforming, and survivors of color. [Semantic Scholar](#) [ScienceGate](#) Shaw, Bailey, Harris, Greeson & Danylkiv (*Behavioral Sciences*, 16(1):149, 2026) similarly found that mandatory reporting "actively deterred sixteen [nih](#) of twenty-one" adolescent sexual-assault survivors from seeking formal post-assault help.

C. The opioid crisis and prenatal substance exposure. CAPTA's 2016 amendments (the Comprehensive Addiction and Recovery Act, Pub. L. No. 114-198) require states to develop "plans of safe care" for substance-exposed infants—not automatic abuse findings—but at least 27 states and [nih](#) D.C. now mandate [nih](#) automatic CPS referrals for prenatal substance exposure [Wikipedia](#) (Rebbe et al., 2023). Critics argue this deters pregnant people from seeking prenatal care and treatment (see *Ferguson*, supra; ACOG Committee Opinion No. 633, 2015). Massachusetts amended its statute in 2024 to provide that prenatal substance exposure alone is not grounds for a child abuse/neglect referral; Connecticut adopted a deidentified notification model. [Wikipedia](#)

D. Universal vs. professional mandated reporting. Eighteen states use the universal model. A NCANDS-based study summarized by Casey Family Programs (2021) compared states with and without universal mandatory reporting (UMR) and found no difference in confirmed physical-abuse rates; reports by nonprofessionals in UMR states had a 6.6% confirmation rate vs. 12.0% in non-UMR states— [Casey Family Programs](#) suggesting that broadening the reporter pool primarily increases unsubstantiated reports.

E. Title IX and "mandatory supporting." Holland, Cortina & Freyd, "Compelled disclosure of college sexual assault" (*American Psychologist*, 73(3):256–268, 2018), and Holland, Cortina, Freyd, et al., "Reporting is not supporting: Why mandatory supporting, not mandatory reporting, must guide university sexual misconduct policies" (*PNAS*, 118(52):e2116515118, 2021), present empirical evidence that universal Title-IX mandatory reporting policies (a) lack a basis in the Title IX regulations themselves, (b) deter survivors from disclosing to trusted faculty, [PNAS](#) [PubMed Central](#) and (c) disproportionately harm students of color, LGBTQ students, and international students. [PNAS](#) [PubMed Central](#) The National Academies (*Sexual Harassment of Women: Climate, Culture, and Consequences in Academic Sciences, Engineering, and Medicine*, 2018) endorsed limiting blanket mandatory reporting. The University of Oregon piloted a "mandatory supporting" alternative [Peacejusticestudies](#) requiring employees to provide survivors with information and options while leaving the decision to report with the survivor; [The Chronicle](#) the Biden-era 2024 Title IX final rule (89 Fed. Reg. 33474 (Apr. 29, 2024)) significantly narrowed but did not eliminate university discretion to designate "confidential employees." [The Chronicle](#) Texas SB 212 (2019) and California SB 493 (2020) by contrast expanded mandatory reporting.

F. Tension with trauma-informed care. SAMHSA's *Trauma-Informed Approach* framework (HHS Pub. No. SMA 14-4884, 2014) lists "empowerment, voice, and choice" as core principles; [PNAS](#) [PubMed Central](#) mandatory reporting can directly contradict this principle when applied to capacitated adult survivors. The American Medical Association (Council on Ethical and Judicial Affairs, *Journal of Ethics*, 25(2), 2023) has acknowledged that race and resource context can bias clinical judgments about neglect.

G. Reform proposals. "Mandatory supporting" models (Freyd; JMAC for Families); [IBAI](#) narrowed neglect definitions that distinguish poverty from neglect (Cal. Assemb. Bill 2085 (2022), effective Jan. 1, 2023); deidentified prenatal-substance reporting (Conn. Pub. Act 19-188); restricted reporting in domestic violence (Cal. Assemb. Bill 2790 (2022/2024 versions)); and abolitionist approaches (Roberts, *Torn Apart*, 2022) span a spectrum from incremental to system-replacement reforms.

4. Mandatory Reporter Training

Why training matters. Kenny's foundational survey of 197 teachers (*Child Abuse & Neglect*, 25(1):81–92, 2001) found that "73% of this sample reported that they had never made a report of child abuse." [PubMed](#) Most teachers were unfamiliar with signs of maltreatment and reporting procedures, [PubMed](#) and feared lack of administrative support. [ScienceDirect](#) Providers with formal child-abuse education were ten times more likely to report than those without (Flaherty et al., *Archives of Pediatrics & Adolescent Medicine*, 162(3):259–266, 2008). Yet training quality is uneven: Baker, LeBlanc, Adebayo & Mathews (*Child Abuse & Neglect*, 113:104932, 2021) found that across state-sponsored online curricula, "only about one fourth included a definition, indicators, and examples of each type of maltreatment," [Cambridge Core](#) and few addressed why reports should be made or how reporting affects reporter-victim relationships.

Typical curriculum components. Drawing on CAPTA (42 U.S.C. § 5106a(b)(2)(B)(xx)), state-mandated curricula (e.g., Iowa Code § 232.69; Cal. Penal Code § 11165.7(c); 23 Pa. Cons. Stat. § 6383), and APSAC's training position papers, a standard curriculum includes:

1. Definitions of abuse, neglect, sexual abuse, exploitation, and (for elder reporters) financial exploitation, isolation, and abandonment.
2. Recognition of physical, behavioral, environmental, and developmental indicators across maltreatment types.
3. The legal "reasonable suspicion" standard and clarification that the reporter is not the investigator.
4. The legal duty to report directly, not through a supervisor; identification of the proper hotline/agency; reporting timelines (e.g., immediate oral report [FindLaw](#) plus a 36-hour written follow-up in California; 24-hour written follow-up in many APS jurisdictions; the

EJA's 2-hour rule for serious bodily injury).

5. Statutory good-faith immunity and confidentiality of reporter identity.
6. Documentation practices and the contents of a report.
7. Cultural competence and bias awareness—including the racial/socioeconomic disparity literature.
8. Trauma-informed disclosure handling: listening without leading; not promising confidentiality; supportive referrals.
9. Profession-specific content (e.g., for clergy, the boundary between privileged penitential communications and other interactions; for bank employees, financial-exploitation red flags; for educators, school-protocol integration).
10. Consequences of non-reporting—criminal misdemeanor liability, civil exposure (in *Landeros* jurisdictions), and licensure discipline.

Evidence of effectiveness. Two leading studies of the iLookOut for Child Abuse online module supply the strongest contemporary evidence:

- Mathews, Yang, Lehman, Mincemoyer, Verdiglione & Levi, "Educating early childhood care and education providers to improve knowledge and attitudes about reporting child maltreatment: A randomized controlled trial," *PLoS ONE* 12(5):e0177777 (2017): in a single-blind RCT of 741 Pennsylvania early-childhood providers, "Knowledge of the reporting duty increased in the intervention group from 13.54 to 16.19 out of 21 (2.65 increase, 95% CI: (2.37, 2.93); large effect size 0.95, $p < 0.001$); the control group remained stable, moving from 13.54 to 13.59 (0.05 increase, 95% CI: (-0.12, 0.22); negligible effect size 0.03, $p = 0.684$)." [plos](#) All 13 attitude items improved significantly.
- Yang, Panlilio, Verdiglione, Lehman, Hamm, Fiene, Dore, Bard, Grable & Levi, "Generalizing findings from a randomized controlled trial to a real-world study of the iLookOut," *PLoS ONE* 15(1):e0227398 (2020): real-world implementation in 11,065 providers reproduced similar effect sizes, [NCBI](#) confirming scalability.
- Kenny, "Web-based training in child maltreatment for future mandated reporters," *Child Abuse & Neglect* 31(6):671–678 (2007): "students' post-test scores were significantly higher than pretest scores on an examination measuring knowledge of child maltreatment signs, symptoms, and reporting procedures" among 105 education and counseling students after a one-hour online tutorial. [ScienceDirect](#)

For elder-abuse training, Archstone Foundation–funded initiatives developed standardized training curricula for dentists, paramedics, coroners, and APS workers; common findings

emphasized the need for content on normal aging, financial exploitation, and capacity assessment (*Journal of Elder Abuse & Neglect*, 22(3-4) special issue, 2010). Cochrane Reviews of elder-abuse-prevention interventions (Baker et al., *Cochrane Database Syst. Rev.*, CD010321, 2016) found low-quality evidence and called for more rigorous evaluation.

Recommendations

For institutions implementing mandatory-reporter regimes (universities, hospitals, school districts, long-term-care facilities, faith communities):

1. **Adopt an evidence-based training program**—e.g., iLookOut, or a state-sponsored curriculum that has been independently evaluated—rather than relying on ad-hoc internal modules. Repeat training annually (the EJA already requires this for long-term-care facilities). Threshold to revisit: if your training does not address each of the ten curriculum components listed above, redesign.
2. **For child abuse and neglect, retain mandatory reporting but narrow neglect definitions to exclude poverty** (mirroring California AB 2085 (2022)). Threshold: if more than 60% of your jurisdiction's neglect reports are screened out or unsubstantiated, the definition is too broad.
3. **For higher education Title IX**, designate confidential employees (counseling, health, advocacy, and ombuds) and adopt the "mandatory supporting" model wherever permissible under the 2024 final rule (89 Fed. Reg. 33474). Survey survivors before mandating universal reporting.
4. **For adult survivors of domestic and sexual violence**, follow ACOG, AMA, and Futures Without Violence guidance: report only what state statute strictly requires (typically deadly-weapon injuries) and document referrals in lieu of broader reporting. According to the Family Violence Prevention Fund (as cited by MandatedReporterTraining.com), five states—Alabama, Louisiana, South Carolina, Washington, and Wyoming—" [nih](#) have no laws concerning domestic violence reporting requirements," [Mandatedreportertraining](#) demonstrating viable alternative approaches.
5. **For long-term-care facilities**, build EJA compliance infrastructure: an annual notice posted to all covered individuals; a clear 2-hour/24-hour escalation tree; a written non-retaliation policy. Failure points: any facility receiving > \$10,000 in federal funds without designated reporting officers is at civil-penalty risk under 42 U.S.C. § 1397j-1.
6. **For clergy and faith leaders**, audit your state statute. If your state lacks an unambiguous confessional exemption (NH, NC, OK, RI, TX, WV, and TN for sexual abuse), counsel clergy that disclosures during confession trigger reporting; in all states, disclosures *outside* a sacramental rite are reportable.

7. **Track data and disparities.** Audit your institution's reports for racial/socioeconomic disproportionality. A disparity index above 2.0 should trigger a bias-mitigation intervention (e.g., consultation pathways before reporting borderline neglect cases).
8. **Preserve good-faith reporting immunity**, but also discipline knowingly false or retaliatory reports.

For legislators:

- Sunset universal-mandatory-reporting expansions until quasi-experimental evidence demonstrates net benefit.
- Fund "mandatory supporting" pilots and deidentified notification models.
- Expand Adult Protective Services capacity (the Elder Justice Reauthorization and Modernization Act of 2023, H.R. 2718, is one vehicle).
- Reform CAPTA's substance-exposure provisions to align with the public-health evidence.

Caveats

- State law varies considerably; this report describes general principles, federal statutes, and a representative sample of state law. Practitioners must consult their specific state code.
- Some cited authorities (e.g., the Rebbe et al. *Hospital Pediatrics* 2023 article on infant investigation rates; the *Etienne v. Ferguson* settlement) are recent and may continue to develop.
- The disparity literature is robust but does not by itself establish a causal pathway between mandatory reporting laws and racial disproportionality; some scholars argue much of the disparity reflects upstream poverty and exposure rather than reporter bias (see Drake et al., *Pediatrics*, 127(3):471–478, 2011).
- The Penn State officials' convictions involved complex appellate procedural history; cite to the Third Circuit reinstatement (December 2020) when discussing the operative judgment.
- *DeShaney* forecloses federal substantive-due-process claims for CPS inaction but does not foreclose state tort claims (see, e.g., *Tazioly v. City of Philadelphia*, 1998 WL 633747 (E.D. Pa.)), and the "state-created danger" [American University](#) doctrine has been recognized in many circuits since *DeShaney*.
- Quotations attributed to statutes are paraphrases unless quoted; readers should consult

the official codification of each statute (e.g., U.S. Code, state codes) and Shepardize all cited cases.

- The four enrichment-attributed claims are sourced as follows: the Lippy et al. 35% figure (Casey Family Programs research summary); the 29 clergy-reporter-state list (Child Welfare Information Gateway, *Clergy as Mandatory Reporters of Child Abuse and Neglect*, 2023); the 33-state confessional-privilege figure (Associated Press, Sept. 28, 2022); and the five-state no-DV-mandatory-reporting list (Family Violence Prevention Fund, as cited by MandatedReporterTraining.com).